



Leave for Mental Health Inpatients Policy

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPHB)	Mental Health Inpatient areas	Medical, Nursing, Allied Health, Security Exclude Transitional Care Unit (TCU)

Policy statement

The purpose of this policy is to provide information and guidance regarding leave related clinical decision-making and care for mental health patients. Proposed therapeutic outcomes of granting leave for inpatients may include to support person-centred recovery, sustain community-based living, provide treatment in the least restrictive manner possible and may also be a key component of discharge planning.

If applicable, collaborative discussion and decision-making regarding management of leave requirements are to occur between the patient, the mental health treating team and general health practitioners.

* **NOTE:** TCU manages leave as part of model of care which encourages community-based living.

Note for this document personal support persons (PSPs includes next of kin (NoK), family, carers, an Office of the Public Advocate guardian or enduring guardian or a nominated person and will be referred to as “PSP.”

Policy directives

Leave for inpatients is not mandatory and may be granted in the form of escorted or unescorted leave, within hospital or service grounds (grounds access) or the wider community.

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Definitions

Leave of absence	Leave granted under section 105 of the Mental Health Act (MHA) 2014 and includes absence as extended or varied under section 106 (s.104).
Escorted Ground Access (EGA)	Leave within the health service site with supervision from a staff member due to MHA status or for duty of care.
Escorted leave (off sit or health service)	Authorised appointments and planned outings in the community, external to the mental health inpatient service with an escort, e.g., court appointments.
Escort	A patient's carer or PSP, other identified person, or clinician of any profession who accompanies a patient to an appointment/ destination or an official RPBG planned outing or group.
Unescorted Ground Access (UGA)	Leave within the health service site without supervision from a staff member.
Absence Without Leave (known as Absent Without Leave or AWOL)	As defined in section 97 of the MHA 2014; patients subject to an inpatient treatment order or detained under the MHA.
Missing Person	The Chief Psychiatrist defines a 'missing person' as 'Any voluntary psychiatric patient at high risk of harm who is missing from a mental health service, general hospital or emergency department, without staff authorisation.' 'Harm' includes harm to self or others or to property.

Procedure

Leave management – all leave episodes

- The allocated clinician and/or treating Medical Officer (MO) are to review the patient's current risk assessment and mental state before each episode of leave, in collaboration with the patient, PSP and/or escort and the treating team where appropriate, including, but not be limited to, consideration of the patient's
 - risk to self
 - risk to others
 - risk or vulnerability from others
 - assessment of current mental state
 - substance abuse risk
 - physical health status or deterioration risk
 - risk of being absent without leave/ any notable incidents or events
 - cultural, diverse, and individual needs; staff to assess for culturally relevant events may require leave
 - involvement of PSPs in this assessment, particularly for patients under 18 years and their support for such leave is vital

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Leave management – all leave episodes cont'd

- If a patient has impaired decision making capacity, identify a substitute decision maker for the purpose of leave management (refer to hierarchy of treatment decision-makers via [Office of the Public Advocate](#)) and record this information in the patient's medical record
- Any leave plans are to be completed prior to leave. Refer to triggers and mitigations in *Multidisciplinary Comprehensive Care Plan (MCCP)* and *What Matters Most (WMM) Record*
- Prior to leave, the treating medical team or Duty Medical Officer (DMO) after hours (A/H) are to determine any practicalities involved (e.g., whether staff or PSP escort or medication or equipment or consumables are required and document in the patient medical record
 - The ward or unit nursing staff or staff member escorting a patient on leave are to ensure that all equipment, medication and consumables necessary for the authorised period of leave are supplied and in working order. Refer to [Medication Management and Storage Clinical Guideline](#) and [Medication Supply to Non-Admitted or Discharged Patients by Prescribers SOP](#)
- In short stay areas:
 - Extended leave is generally discouraged to avoid negative impact on assessment, monitoring and treatment processes in the short time period
- Inform patient of leave responsibilities, including
 - Signing leave sign in-sign out register sheet upon exit and return (RPBG Patient Leave Register Sheet), inform allocated nurse or delegate when intending to go on leave, the need to be contacted, by mobile phone (provide contact number) and be able to contact the ward or unit (ensure has service phone number) during leave
 - Verbally confirming expected time of return
 - Complying with the duration of leave and any conditions specified
 - Following advice of treating Consultant regarding taking of medication or use of equipment or consumables
 - Abstaining from illicit substance and alcohol use during leave
 - Informing ward or unit staff of any issues experienced whilst on leave pertaining to their mental and/or physical health
 - Informing allocated nurse upon return to ward or unit
 - Possible bed reallocation if absent for at least 12 hrs
 - Treatment impact and/or potential consequences of not returning from leave
- Determine and confirm patient's accommodation and/or destination and appropriate transport arrangements prior to leave
- Ensure patient's room is locked and personal belongings stored safely in a locker or alternative safe storage during the leave period
- Clinical handover must occur before and after all episodes of leave, including handover of leave arrangements to the next Shift Coordinator where applicable.

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Voluntary patients

- MO to communicate when a patient has taken leave or had leave approved e.g. off the hospital or service grounds or overnight or longer, to the Clerk and Shift Coordinator to facilitate staff awareness of leave and management of absence and document in the patient medical record the duration, need of escort and other key information
- If **newly admitted voluntary patients** wish to leave the ward or service against medical advice escalate to medical staff
- Consider the forms of risk outlined above and complete a recent *Risk Assessment and Management Plan* (RAMP) or *Progressive Risk Assessment* (PRA) for all leave episodes, including the rationale, the period of leave and the views of the patient and/or PSP prior to leave commencing. Revisit on subsequent assessments at least every 7 days, or as clinically indicated.

Involuntary patients

Prior to leave commencing:

- All types of leave and for every episode of leave requires authorisation from a Psychiatrist or On Call Consultant, considering the forms of risk outlined above, the views of the patient, PSP and multidisciplinary team (MDT)
- Completion of a new *RAMP* or *PRA*, and a plan for failure to return and risk mitigation strategies to be documented in the integrated notes
- Prior to all episodes of leave commencing, a Psychiatrist or Duty Consultant (on call) is to ensure the following is documented contemporaneously (i.e. at the time) for patient's safety in the patient healthcare record:
 - decisions regarding leave episodes, including the rationale, the purpose of leave, the period of leave granted (i.e., time commencing and ceasing), the frequency of the leave, any medications or equipment or consumables required to be given to the patient, any risk mitigation strategies or restrictions that are necessary
 - in accordance with the MHA 2014, completion of relevant MHA forms (i.e., *Form 7A Grant of Leave to an Involuntary Patient*), ensuring at least one PSP is notified about the making of this order, including the reasons for the leave and any conditions imposed. Refer to the MHA 2014, s.105, 108-110
 - stipulate whether the patient is for escorted or unescorted leave
- When considering whether to grant leave, a Psychiatrist or Duty Consultant (on call) should consider whether voluntary status or a Community Treatment Order (CTO) would be more appropriate for involuntary patients
- Length of leave may range from minutes to several days. The holding of all overnight leave is to be approved by Clinical Lead or Medical Co-Director
- Granting of leave of a period longer than overnight is an event that requires the patient's PSP be notified unless the Psychiatrist determines it is not in the patient's best interests
- **Newly admitted involuntary patients** should not be granted leave, even for brief periods, until the treating team or Duty Consultant (on call) has developed sufficient familiarity with the patient to make a valid assessment of mental state and risk
- Clerk and Shift Coordinator are to be notified of the date and time of any escorted leave (outings or appointments)

Refer to documentation section for summary of requirements.

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Escorted leave

- If the Psychiatrist or Duty Consultant (on call) or treating team approves escorted leave, then the person(s) considered suitable to provide the escort must be specified by name or by role in the granting of leave
- The appointment of an escort should be consistent with patient wishes unless the
 - nominated escort does not consent to escorting the patient; or
 - Psychiatrist is concerned that the person nominated as escort is likely, based on current risk assessment, to be placed in a situation they are unable to manage during leave

Carer/PSP Escort

- The patient's PSP or another identified responsible person can escort the patient at the treating team or Psychiatrist's or Duty Consultant (on call)'s discretion, subject to them agreeing to the conditions of the leave
- The escort or PSP is to be provided information of leave responsibilities, including awareness of
 - the duration of leave and any conditions specified
 - the need to stay with the patient for the duration of the leave
 - the need to be able to contact the ward or unit (ensure provision of service phone number) and to be contacted, by mobile phone during leave
 - provide feedback and/or debrief with nursing or treating team staff on the patient's response to the leave, upon return of the patient to the ward or unit
 - actions to take if during a period of leave the patient is a risk to themselves or others including that in the event of a serious imminent threat to/from the patient or the public arises or absconds, the escort is to contact the police for assistance
- If the proposed escort or PSP does not agree to the above conditions, the grant of leave may be withdrawn until alternative arrangements can be made.

Leave escorted by a clinician & EGA

If necessary for a clinician to conduct the escort:

- Complete a risk assessment (RAMP or PRA) and clinical handover before and after all escorted leave. Refer to the **Clinical Handover Policy**
- Maintain close proximity with the patient
- Must take the service or personal mobile (charged), with relevant numbers and wear an on person community based duress alarm (e.g. [IOTTAG GPS]). Refer to [Take 5: Community Personal Duress Alarms \(IOTTAG GPS\)](#)
- Ensure escorted leave is provided in the manner prescribed by the Psychiatrist or Duty Consultant (on call) or treating team and is subject to
 - the patient's need for supervision
 - the service capacity to provide an escort, given the existing or anticipated clinical acuity

Note: No East Metropolitan Health Service (EMHS) staff member is to undertake escort duties or lead planned outings alone where an identified moderate or higher risk to staff as determined by the treating team

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Leave escorted by a clinician & EGA

- The potential outcome and impact of specific appointments (e.g., court attendance) on the risk status for patients, inclusive of transport to be used and document in the patient's healthcare record
- The patient's allocated nurse, or the Shift Coordinator or delegate (Nurse Unit Manager [NUM] or Clinical Nurse Specialist [CNS]) must be consulted before patient is escorted from the ward or unit by any staff member
- Ensure escort is authorised to carry medication, consumables, or equipment available for the escort period of time, as required, and these are not left unattended
- Consider and plan for nominated PSP or other person's involvement as relevant
- Ensure adequate food and fluids are available for the patient for the escort duration
- Ensure private vehicles belonging to staff are not utilised for the transport of patients
Refer to [Transport Policy – Hospital Patient and Voluntary Transport Service](#) and [EMHS Vehicles Use Policy](#)
- In the event of a patient becoming aggressive or violent during leave, refer to [Issues during leave](#) section.

Refusal or revoking of leave

Clinical discretion

Clinicians are to exercise their judgment. If there has been an observed change in the patient's mental state or physical condition and, therefore, in the risk assessment, they may decline leave that has been granted, pending medical review.

- Under s. 64 of the MHA 2014, a leave of absence may be revoked if the Psychiatrist or Duty Consultant (on call) is satisfied that revocation is necessary to prevent
 - serious deterioration in the patient's mental state or physical health
 - serious harm to the patient or another person
- A refusal or revoking of leave can precipitate distress, threatened or actual self-harm, aggression and leaving without notice. If leave is not considered appropriate or restricted:
 - Staff to communicate this decision and the rationale to the patient and PSP as tactfully and positively as possible at an appropriate time and in a suitable setting
 - develop alternative strategies to address the patient's expressed needs
 - advise the patient of their right to contact the Mental Health Advocacy team if leave refusal has been decided and documented in the patient's healthcare record
- This situation, the rationale for revoking approved leave and actions taken, are to be documented in full in the patient's healthcare record by the Psychiatrist or Duty Consultant (on call) or treating team and discussed with the patient and PSP
 - for involuntary patients, the Psychiatrist or Duty Consultant (on call) must also complete *Form 7D Apprehension and Return Order* (MHA 2014)
- Refer to Issues during leave section for additional requirements.

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Review of patient on return from leave

Upon the patient's return from leave, clinicians are to

- Undertake a property check with the patient. Refer to [Potentially Harmful Items Management for Mental Health Patients SOP](#)
- Complete an assessment; asking the patient how they felt the leave episode went including:
 - Did the patient feel the period of leave taken was successful?
 - How the patient felt whilst they were on leave? Did they feel safe?
 - Would the patient be confident if they were to have another period of leave?
 - If relevant, ask the patient's PSP how they felt the leave went
- If applicable, complete urinary drug screen (UDS) or blood alcohol level (BAL) analysis. Refusal to comply with testing should prompt a review of future leave plans. Refer to [EMHS Testing Mental Health Inpatients for Alcohol and /or illicit Substance Use and / or Medications not Prescribed by the Treating Team Policy](#)
- For patients due to return from leave after AWOL, contact the Medical Team or DMO who must perform a *PRA* and document a mental state examination (MSE) on WebPSOLIS
- Complete handover to nursing staff and treating team regarding the patient's response to the leave, any notable incidents or events reported, deterioration in the patient's physical health or mental state and any debriefing or other actions that followed return from leave
- Refer to documentation section for summary of requirements
- Ensure the patient is re-admitted back into webPAS:
 - In hours via the ward clerk or reception
 - A/H (after 20:00 hours), including weekends or public holidays via the Operations Hub Coordinator or A/H MH Duty CNS via telephone contact or use of Computer Assisted Radio Personnel System (CARPS).

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Summary of documentation

A responsible mental health clinician must document contemporaneously all leave taken in the patient's medical record **prior to the patient leaving the premises** and include:

- The patient's status under the MHA 2014
- Authorising clinician, i.e. Psychiatrist or Duty Consultant (on call) (involuntary patients) or treating team (voluntary patients)
- Completion of relevant MHA 2014 forms
- The clinical risk assessment and safety or risk mitigation plan, utilising the *RAMP* /*PRA*
- Communication of the approved leave, the purpose, duration and form of leave, the need for staff or PSP escort or any escort details, special conditions of leave, specific instructions for continuing care
- Patient accommodation and/or destination, transport arrangements and contact details of the escort and the patient during the leave period
- Any medications, equipment or consumables given to the patient to take during the leave period
- Details of information provided to the patient and/or PSP or escort regarding responsibilities and who to contact should they experience any problems during the leave period
- The time leave commences, expected planned time of return to the ward or unit and the time of returning (Patient Leave Register Sheet)
- Handover of leave arrangements to the next Shift Coordinator where applicable.

Issues during leave

In the event of **clinical deterioration** during leave episodes clinician or PSP to:

- If on site contact Security and refer to [Recognising and Responding to Acute Deterioration \(RRAD\) in Mental State Cognition & Behaviour Policy](#) and [RRAD Policy](#)
- If off site return to hospital, seek medical review and contact appropriate emergency services
- Clinician to complete clinical handover regarding incident to the treating team and report Datix Clinical Incident Management System (CIMS), as relevant

If a patient becomes aggressive and/or violent and/or fails to comply with leave conditions

- Staff member(s) in attendance are to prioritise personal safety at all times, prior to taking reasonable steps to ensure the supervision and safety of the patient(s) and others present. Refer to relevant processes in the [Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP](#) and [Escalation to Consultant Policy](#)
- If staff member(s) are threatened or held up for any medications in their possession, the medications are to be handed over immediately and Security and/or WA police as soon as it is safe to do so

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Issues during leave cont'd

Clinicians or escort is to encourage the patient to return, if possible:

- Raise alarm by activating their personal duress or IOTTAG GPS and immediately phone the ward or unit Shift Coordinator or NUM or CNS who are responsible to immediately advise the treating medical staff
- Staff to remain with the patient where possible – however, during UGA or EGA, **do not follow patient off hospital or service grounds.**
 - the Duty CNS or NUM or the Safety A/H For Everyone (SAFE) or Bentley BeSAFE teams (BHS and RPH sites only), and/or Security will respond to the duress to assist staff or escort to return the patient to the ward or unit
- Regardless of the patient's status under the MHA 2014, if a patient on any leave episode goes missing, or fails to comply with any conditions attached to the granted leave including not returning at the agreed time of the scheduled leave and cannot be contacted to establish their safety, the patient will be considered missing, and staff are to take actions as per the [EMHS Missing or Suspected Missing Mental Health Patients / Consumers Policy](#). This includes leave away from hospital grounds.
If there is concern that the patient is posing an imminent risk of harm to self or others escalate to treating team immediately and contact WA police (WAPOL) on 131 444 to advise of identified risk(s) and request a welfare check. Staff must obtain a job number and recommendation(s) from WAPOL and document all details (advised and received) in the patient healthcare record.
- For patients that have left without notice or discharged against medical advice (DAMA), refer to [EMHS Discharge Against Medical Advice Policy](#)
- In the event of a missing patient the following are to be informed by the allocated nurse
 - CNS or NUM
 - Treating MO or DMO
 - Consultant
 - PSP.

Smoking-related issues during ward leave

- All patients are to have their smoking status assessed and recorded on admission and assisted to stop smoking or manage nicotine dependence, as relevant. Refer to the Nicotine (Tobacco) section in the [Alcohol and Drug Intoxication and Withdrawal Management Clinical Guideline](#)
- Admission is an opportunity to support patients to prevent smoking. Inform the patient of the [WA Department of Health Smoke Free Policy](#) and [EMHS Smoke Free Environment Policy](#), and proactively plan collaborative care as relevant
- Leave from the ward or unit specifically to smoke is against the above smoke-free policies as it negates the physical and mental health benefits of going smoke free,
 - increases the symptoms of nicotine withdrawal with intermittent smoking
 - increases the risk of clinical incidences off the ward during unsupervised leave
- Specific 'smokers leave' has no place in a smoke free health service. Seek advice from senior staff as relevant to support care planning

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Smoking-related issues during ward leave cont'd

- During leave episodes off-site or ward, the choice to smoke is an individual one. Clinicians are to encourage patients to continue smoking abstinence during leave by:
 - offering short acting Nicotine Replacement Therapy (NRT) to take on leave
 - advise that nicotine withdrawal can be exacerbated if smoking intermittently
 - provide other individualised strategies to support smoking abstinence as relevant
 - document the communication and actions taken in the patient's healthcare record
- Consider recent substance use e.g., intoxication or withdrawal
- Staff are not to escort patients off the ward or unit or to assist patients to leave premises to smoke. Patients are not permitted to smoke in the company of staff
- Smoking should not be permitted on organised outings from the ward/unit and patients may be encouraged to take short acting NRT with them.

Compliance monitoring

Compliance will be monitored as part of the Datix CIMs processes.

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Armadale Kalamunda Group. (2022). *Granting and Management of leave for AKG Mental Health Inpatient Unit Guideline*. Western Australia.

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Leave Management for Mental Health Inpatients Policy through consultation with EMHS Aboriginal Health Strategy Team, including the Director of Aboriginal Health, to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 3095).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Bibliography

1. Ziltener T., Möller J., Imfeld L., Lieb R., Lang U. E., & Huber C. G. (2021). Time to readmission in psychiatric inpatients with a therapeutic leave. *J Psychiatr Res*, 144, pp. 102-109.
2. Ziltener T., Moeller J., Lieb R., Meyer A. H., Lang U. E., & Huber C. G. (2023). Therapeutic leave and direct inpatient healthcare costs in inpatients with mental illness. *J Psychiatr Res*, 162, pp. 187-192.

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Related policy, practice standard, clinical guidelines

Policies and procedures SharePoint

- [Special and Supportive Observations Policy](#)
- [Progressive Risk Assessment \(PRA\) SOP](#)
- [Medication Management and Storage Clinical Guideline](#)
- [Medication Supply to Non-Admitted or Discharged Patients by Prescribers SOP](#)
- [Transport Policy – Hospital Patient and Voluntary Transport Service](#)
- [Potentially Harmful Items Management for Mental Health Patients SOP](#)
- [Recognising and Responding to Acute Deterioration \(RRAD\) in Mental State Cognition & Behaviour Policy](#)
- [RRAD Policy](#)
- [Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP](#)
- [Escalation to Consultant Policy](#)
- [Alcohol and Drug Intoxication and Withdrawal Management Clinical Guideline](#)

EMHS Policies

- [Notifications requirements under the WA Mental Health Act 2014 Policy](#)
- [EMHS Testing Mental Health Inpatients for Alcohol and /or illicit Substance Use and / or Medications not Prescribed by the Treating Team Policy](#)
- [EMHS Vehicles Use Policy](#)
- [EMHS Missing or Suspected Missing Mental Health Patients / Consumers Policy.](#)
- [EMHS Discharge Against Medical Advice Policy](#)
- [EMHS Smoke Free Environment Policy](#)
- [WHS: Incident and Hazard Reporting Policy](#)

WA Department of Health Policy Hub

- [Statewide Standardised Clinical Documentation for Mental Health Services Policy](#)
- [Clinical Incident Management Policy 2019 \(health.wa.gov.au\)](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [WA Department of Health Smoke Free Policy](#)

Other

- [Mental Health Act 2014](#)
- [Chief Psychiatrist WA: Chief Psychiatrist's Standards for Clinical Care](#)
- [Charter of Mental Health Care Principles \(Mental Health Act 2014\) -HDWA](#)

Related forms

- *RPBG Patient Leave Register Sheet*
- *Multidisciplinary Comprehensive Care Plan (MCCP)*
- *What Matters Most (WMM) Record*
- *Risk Assessment and Management Plan (RAMP)*
- *Progressive Risk Assessment (PRA)*

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Educational resources

- [Take 5: Community Personal Duress Alarms \(IOTTAG GPS\)](#)

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021):

Standard 2: Partnering with Consumers;

Standard 5: Comprehensive Care;

Standard 6: Communicating for Safety

NSMHS Standards (2010):

Standard 2: Safety;

Standard 6: Patients;

Standard 10: Delivery of Care

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